Behavioral Health Policy Advocacy in California: A Strategic Framework

Translating Proposition 1 and the CARE Act into the "Handcuffs Aren't Housing" Toolkit

The behavioral health landscape in California is currently undergoing its most significant structural transformation since the passage of the Mental Health Services Act (MHSA) in 2004. This shift is characterized by a fundamental move away from the "Housing First" and voluntary, peer-led recovery models toward a more coercive, institutionalized system of state-mandated intervention.[1] This evolution is driven by a suite of legislative mandates: most notably Proposition 1, the Community Assistance, Recovery, and Empowerment (CARE) Act, and Senate Bill 43; which together reorganize how billions of dollars are allocated and how the state interacts with individuals experiencing severe mental illness and homelessness.[3] The "Handcuffs Aren't Housing" advocacy toolkit represents a critical intervention in this policy shift, seeking to illuminate the systemic risks of mass institutionalization, the erosion of civil liberties, and the disproportionate impact of these policies on marginalized communities.[6]

The Fiscal Reengineering of California's Behavioral Health System

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The passage of Proposition 1 in March 2024 fundamentally altered the fiscal architecture of California's mental health system by replacing the Mental Health Services Act (MHSA) with the Behavioral Health Services Act (BHSA).[3] For two decades, the MHSA served as a dedicated funding stream, funded by a 1% tax on personal income over $1 million, that allowed counties significant flexibility to provide community-based services, prevention programs, and innovative pilots.[2] The BHSA, however, introduces rigid spending mandates

that prioritize capital infrastructure and housing interventions over existing outpatient services.[1]

The restructuring creates a fiscal environment where counties must now allocate 30% of their funds specifically for housing interventions, 35% for Full Service Partnerships (FSP), and 35% for a consolidated category of behavioral health services and supports (BHSS).[1] While the rhetoric surrounding Proposition 1 emphasizes the creation of new housing, the reality is a zero-sum funding environment. Because the BHSA does not increase the overall amount of revenue collected through the millionaire's tax, the new mandates force counties to divert resources away from voluntary, peer-run, and community-based programs that have historically been funded through the MHSA's Prevention and Early Intervention (PEI) and Innovation categories.[7]

Comparative Fiscal Analysis: MHSA vs. BHSA Funding Allocations

Spending Category	MHSA Allocation (Pre-Prop 1)	BHSA Allocation (Post-Prop 1)	Targeted Population/Primary Focus
Community Services & Supports (CSS)	76%	N/A	Broad outpatient services and outreach.[8]
Prevention & Early Intervention (PEI)	19%	N/A	Early detection and crisis prevention.[8]
Innovation	5%	N/A	Pilot programs and experimental care models.[8]
Housing Interventions	Local Discretion	30% (Mandatory)	Chronically homeless with SMI/SUD.[3]
Full Service Partnerships (FSP)	Included in CSS	35% (Mandatory)	"Whatever It Takes" intensive support.[1]
BHSS (Services &	N/A	35% (Consolidated)	Workforce, capital,

Supports)			and early intervention.[8]
State Administrative Share	~5%	~10%	Redirects ~$140M annually to the state.[3]

The implications of this shift are profound for peer-run organizations. Projections suggest that the new regulations could lead to a 30% to 50% reduction in voluntary outpatient and community-based culturally responsive treatment programs.[7] This funding gap is exacerbated by the fact that the BHSA also redirects approximately $140 million annually from local counties to state-level administration, further diminishing the resources available for direct community care.[3] As counties transition to this new model, they are required to develop Integrated Plans for the 2026-2029 fiscal years, a process that represents a critical window for advocates to challenge the gutting of voluntary services.[3]

The CARE Act and the Mechanisms of Judicial Intervention

The Community Assistance, Recovery, and Empowerment (CARE) Act (SB 1338) and its subsequent expansion (SB 27) represent the state's move toward a judicial model of behavioral health management.[4] The CARE Court system is designed to provide a pathway for family members, first responders, and clinicians to petition a civil court to order individuals with schizophrenia or other psychotic disorders into treatment.[5] Proponents of the CARE Act argue that it provides necessary accountability for individuals who have "fallen through the cracks" of the voluntary system.[13] However, advocates and civil rights organizations, including Disability Rights California (DRC), view it as a coercive tool that undermines autonomy and relies on stigma rather than proven clinical outcomes.[12]

Implementation data from the first phase of CARE Court highlights a significant disconnect between state projections and reality. While the administration estimated that between 7,000 and 12,000 Californians would qualify for the program, actual petition numbers and established agreements have been significantly lower.[14] The high dismissal rates in major

urban centers such as Los Angeles and San Francisco suggest that many individuals being swept into the system do not meet the narrow clinical criteria or are already engaged in other forms of care that the court cannot effectively supplement.[11]

CARE Court Implementation Metrics (Selected Jurisdictions, 2024-2025)

Metric	Los Angeles County	San Diego County	Sacramento County	Statewide (Aggregate)
Petitions Filed	511	384	74	2,421
Agreements/Plans Established	112	134	25	528
Dismissal Rate	~78%	~65%	~33%	~45%
Total Graduations	0	(Limited)	(In Progress)	(Negligible)
Initial Estimated Need	4,500	1,000	N/A	7,000 - 12,000

Data compiled from.[11]

The expansion of the CARE Act under SB 27, which now includes individuals with bipolar disorder and allows for direct referrals from the criminal justice system, indicates a legislative trend toward broadening the reach of the judicial net despite the initial program's struggles to achieve its stated goals.[11] For advocates, the CARE Act serves as the third step in the "Coercion Trap," where state-mandated oversight becomes the primary gatekeeper to services, replacing the principle of self-determination with a court-approved CARE plan.[6]

SB 43 and the Redefinition of Grave Disability

Parallel to the fiscal and judicial shifts, Senate Bill 43 has fundamentally altered the legal standards for involuntary commitment in California.[5] By expanding the definition of "Grave Disability" under the Lanterman-Petris-Short (LPS) Act, SB 43 allows for the involuntary detention of individuals who are unable to provide for their "personal safety" or "necessary medical care" due to a mental health disorder or a severe substance use disorder (SUD).[5] This represents a significant departure from the previous standard, which was limited to the inability to provide for food, clothing, or shelter.[5]

The inclusion of SUD as a qualifying condition for involuntary holds (5150) is particularly controversial. It shifts the state's approach to addiction from a public health and harm reduction model to one of carceral intervention.[16] Peace officers and county-designated personnel are now tasked with making field determinations about whether a person's substance use has rendered them unable to attend to their safety, a standard that critics argue is dangerously vague and prone to subjective bias.[5]

Clinical and Legal Criteria for Involuntary Holds under SB 43

New Statutory Elements	Observable Functional Impairments	Professional Requirements
Severe Substance Use Disorder	Overwhelming odor, unsteady gait, slurred speech, patterns of overdose.[16]	Determination by authorized professional or peace officer.[5]
Personal Safety	Wandering into traffic, extreme hoarding, inability to use appliances safely.[16]	Connection to SMI/SUD must be established for detention.[19]
Necessary Medical Care	Refusal of care for serious injury, untreated life-threatening comorbidities.[5]	Determination that untreated condition likely leads to serious injury.[5]
Expanded Testimony	Broader evidence allowed in conservatorship	Prioritizes clinical history over current state in some

	proceedings.[18]	cases.[18]

The expansion of these criteria creates a "closed loop" of institutionalization. By lowering the bar for what constitutes a behavioral health emergency, the state effectively increases the volume of individuals entering psychiatric hospitals and jails, while simultaneously defunding the diversion programs that could help them transition back to community-based life.[6] This mechanism ensures that the "solution" to homelessness becomes a revolving door of locked wards and emergency departments rather than stable, permanent housing.[6]

The Racial Justice Crisis and the "Whiting" of California's Streets

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The "Handcuffs Aren't Housing" framework posits that the push for forced treatment is not merely a mental health policy issue but a profound racial justice crisis.[6] Advocacy materials argue that the systemic "Whiting" of California's streets involves the dehumanization and erasure of poor people and people of color by sequestering them in psychiatric institutions and jails.[6] This argument is grounded in the well-documented phenomenon of diagnostic bias, where Black and Brown Californians are disproportionately diagnosed with schizophrenia; the primary diagnostic "key" used to unlock the coercion pipeline.[6]

Research consistently indicates that African Americans are three to four times more likely to be diagnosed with schizophrenia than their white counterparts, even when presenting with similar symptoms.[20] This disparity is not rooted in genetic differences but in clinical bias, where practitioners are more likely to interpret mood symptoms in Black patients as signs of psychosis.[21] Because CARE Court and the expanded LPS holds specifically target individuals with psychotic-spectrum disorders, these policies inevitably function as a dragnet for communities of color.[12]

Racial Disparities in Schizophrenia Diagnosis and Behavioral Health Access

Demographic Group	Schizophrenia Diagnosis Rate vs. Whites	Access to Outpatient Psychotherapy	Medi-Cal/Insurance Trends
African American	3.0x - 4.0x Higher [20]	Significantly Lower [23]	48% with schizoaffective disorder have Medicaid.[22]
Latino/Hispanic	~3.0x Higher [20]	Lower (75% of White rates) [23]	Persistent disparities in specialty care access.[23]
Native American	Highest Overall MH Diagnosis [24]	Highly Variable	Targeted by specific grant cuts.[24]
White (Non-Hispanic)	Baseline (1.0)	Highest [24]	Most likely to receive consistent medication/therapy.[24]

This diagnostic bias means that Black Californians, who already make up 40% of the state's unhoused population, are at an exponentially higher risk of being subjected to the CARE Act or SB 43.[12] The "Handcuffs Aren't Housing" toolkit urges advocates to call this what it is: a move to disappear the poor and people of color behind bars and hospital beds.[6] The political hypocrisy of California's leadership, claiming to be the "anti-Trump" state while adopting dehumanizing tactics of mass institutionalization, is a central theme in the call for racial equity and homes, not handcuffs.[6]

The Evidence for Peer-Run and Voluntary Services

Central to the CAMHPRO advocacy strategy is the assertion that policymakers are ignoring robust data in favor of anecdotal politics.[6] Peer-run services, which are led by individuals

with lived experience of mental health challenges and recovery, have been shown to lead to higher stability, fewer crises, and genuine recovery compared to forced treatment models.[6] Programs like NAMI's "Next Steps" and "PeerLINKS" in San Diego County have successfully reduced hospital readmissions by bridging the gap between inpatient treatment and community-based recovery.[26]

Despite the effectiveness of these models, they are currently facing a "double-jeopardy" situation. While Proposition 1 shifts funding away from the grassroots organizations that provide these services, federal cuts, such as the abrupt termination of SAMHSA grants, are further destabilizing the community-based care network.[25] Advocates argue that instead of funding $6.4 billion in bonds for locked facilities, the state should be protecting and expanding the funding for voluntary, peer-led services that have a proven track record of success.[6]

Efficacy and Outcomes of Peer-Led vs. Coercive Intervention

Service Model	Key Outcome Metrics	Human Rights Perspective	Fiscal Efficiency
Peer Respites	High consumer satisfaction; reduced ED visits.[27]	Centers autonomy and self-determination.[28]	High (Low cost per encounter).[28]
Full Service Partnerships	Reductions in homelessness and incarceration.[3]	"Whatever It Takes" supportive approach.[1]	High ROI for long-term stability.[1]
Involuntary Holds (5150)	Short-term safety; high trauma risk.[5]	Strips basic civil liberties.[7]	Low (High cost for short-term fixes).[7]
CARE Court	Coordination but not direct care provision.[12]	Coerced participation via judicial oversight.[12]	Very Low (High administrative overhead).[12]

The data Notebook on Wellness and Recovery Centers indicates that 81% of supervisors in

these peer-led programs have lived experience, and 77% employ certified peer support specialists.[27] These programs are designed to address the spirit, body, mind, and community in a holistic manner, emphasizing trauma-informed and culturally relevant care.[28] When the state prioritizes "handcuffs" (coercion) over "housing" (voluntary support), it effectively dismantles the very infrastructure that prevents the crises it claims to solve.[6]

Building the "Handcuffs Aren't Housing" Advocacy Toolkit

For a statewide advocacy toolkit to be effective, and for it to serve as a professional asset for a policy recruiter, it must transcend simple flyers and incorporate a comprehensive strategy based on the best practices of government relations and public affairs.[29] A high-quality toolkit serves three core functions: it educates supporters on the issues, equips them with consistent language and materials, and activates them to take direct action with decision-makers.[31]

Strategic Components of a Professional Advocacy Toolkit

The following framework adapts my approach in drafting materials into a professional structure that demonstrates mastery of the policy landscape.

1. Background and Issue Education (The Narrative)

The toolkit must clearly define the "Coercion Pipeline" using the four-step "Closed Loop" model provided in the draft.[6] This narrative should be supported by the statistics on racial diagnostic bias and the projected 30-50% cuts to voluntary services under Proposition 1.[7] By framing the issue as a "Racial Justice Crisis" and "Political Hypocrisy," the toolkit creates an emotional and logical imperative for action.[6]

2. Messaging and Power Statements (The Script)

Consistent messaging is the foundation of any successful advocacy campaign.[30] The toolkit should provide "Power Statements" tailored for different audiences, such as County Boards of Supervisors or state assembly members.[6] These statements must emphasize the local relevance of the issue, using "I" statements and citing specific examples of successful local peer programs that are now at risk of defunding.[34]

3. Action Items and Call to Action (The Mobilization)

Advocacy is about reducing friction between intent and action.[31] The toolkit should include:

- **A "Find Your Meeting" Guide:** Clear instructions on the "Tuesday Rule" and links to county directories.[6]
- **Sample Letters to Lawmakers:** Pre-researched templates that supporters can personalize to express support for voluntary care and opposition to coercive funding shifts.[34]
- **SMART Advocacy Objectives:** Specific goals, such as "Retain 100% of peer-run funding in the upcoming County Integrated Plan".[33]

4. Visual and Media Assets (The Branding)

Professional designs that utilize consistent color schemes, high-contrast typography, and clear calls to action are essential for credibility.[32] Social media graphics should be provided in multiple formats (square for Instagram, landscape for LinkedIn/X) to ensure the message can be spread across various digital platforms.[31] Impactful storytelling, using images and testimonials from individuals who have thrived in peer-led recovery, is more effective than dry statistics alone.[32]

Structural Layout for a Professional Policy Portfolio Toolkit

Component	Content Detail	Purpose for

		Recruiter/Employer
Issue Briefs	1-2 page summaries of BHSA and CARE Court impact.[31]	Demonstrates analytical depth and policy expertise.
Stakeholder Map	Identified decision-makers in the Board of Supervisors.[30]	Shows strategic thinking and networking capability.
Media Kit	Sample press releases and social media graphics.[31]	Displays communication and branding skills.
Action Alerts	Templates for mobilizing grassroots supporters.[34]	Exhibits campaign management and leadership.
Evaluation Plan	KPIs to track legislative interactions and media mentions.[31]	Proves a results-oriented, data-driven approach.

Actionable Steps for Statewide Advocacy: Engaging the Board of Supervisors

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The most critical venue for advocacy over the next 24 months will be the County Board of Supervisors.[3] As counties develop their Integrated Plans for the rollout of the BHSA in 2026, they are required by law to engage in a Community Planning Process that includes input from individuals with lived experience and their families.[10] This is the opportunity to demand that the "Innovation" and "PEI" values of the MHSA are not lost in the transition to the BHSA.[10]

Navigating the "Tuesday Rule" and Public Comment

Advocates are encouraged to show up to Board of Supervisors meetings, most of which occur on Tuesday mornings, to provide public comment during sessions related to "Health

and Social Services" or "Behavioral Health Budgets".[6] Public comment should be focused on the "Ask": that the Board protects the funding for voluntary, peer-led services and stops the expansion of coercive practices that lead to trauma and the revolving door of institutionalization.[6]

Step-by-Step Engagement Protocol:

1. **Locate the Agenda:** Use the county directory to find the upcoming board calendar and search for "Behavioral Health Services Act" or "Integrated Plan".[6]
2. **Prepare the Power Statement:** Select one of the four draft power statements (Racial Justice, Political Hypocrisy, Closing Pipeline, or Peer-Run Data) and personalize it with local data.[6]
3. **Submit Written Testimony:** In addition to verbal comment, submit a formal letter to the board to ensure your concerns are part of the official public record.[34]
4. **Network with Allies:** Join monthly CAMHPRO advocacy meetings, such as the "Opposing Coercive Treatment" group held on the first Tuesday of every month, to coordinate messaging with other organizations.[28]

Future Outlook: Reclaiming the Narrative of Care

The shift toward the "Handcuffs Aren't Housing" model is not inevitable; it is the result of specific policy choices that prioritize institutional beds over human dignity.[6] As the state launches new "accountability" measures to speed up the implementation of the CARE Act and Proposition 1, the role of advocacy becomes even more essential.[13] These accountability tools often involve withholding funding from local governments that do not demonstrate "progress," a metric often defined by the number of petitions filed or beds created, rather than the number of lives stabilized through voluntary care.[13]

The next several years will be defined by the struggle to preserve the "Whatever It Takes" philosophy of the Full Service Partnerships while fighting against the gutting of the peer-run organizations that make that philosophy a reality.[1] By translating complex legislative mandates into an accessible, actionable, and professionally designed toolkit,

advocates can empower their communities to demand a behavioral health system that offers homes, not handcuffs, and care, not coercion.[6]

Conclusion: A Strategic Path Forward

The "Handcuffs Aren't Housing" advocacy framework provides a robust and necessary counter-narrative to the current legislative trend of mass institutionalization in California.[6] By synthesizing the fiscal changes of Proposition 1, the judicial mechanisms of the CARE Act, and the expanded detention criteria of SB 43, the toolkit highlights the systemic risks of a "force-first" approach to mental health and homelessness.[5] For the professional seeking to demonstrate their expertise to recruiters, the creation of such a toolkit is an exercise in high-level policy analysis, strategic communication, and social justice leadership.[31]

Success in this advocacy space requires a dual focus: maintaining a clear, emotionally resonant message about racial justice and civil liberties, while simultaneously mastering the technical details of county integrated plans and state-level funding shifts.[3] By utilizing the "Power Statements" and local engagement strategies outlined in this report, advocates can work to ensure that California's behavioral health system remains committed to the principles of recovery, equity, and voluntary care.[6] The window for change is closing, but with a well-structured and data-driven toolkit, the community has the power to reshape the legacy of California's behavioral health future.[6]

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